PROGRAM PROPOSAL · DRAFT FOR DISCUSSION

Bringing Zendo Home: East Coast Expansion & Community Crisis Response

A phased, chapter-based plan to extend Zendo Project's compassionate psychedelic peer support from festivals and events into year-round, neighborhood-level crisis response — beginning with a single pilot chapter in Brooklyn, New York.



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Prepared for: Zendo Project leadership, MAPS, and prospective partners
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1. Executive summary

For more than a decade, Zendo Project has shown that a calm space and a trained, compassionate peer can help people move through frightening psychedelic and emotional experiences — often without police, sedation, or a hospital visit. This proposal asks: what would it look like to offer that same care where people live, all year round?

The idea

Create a **Zendo Community Crisis Response program** organized as small, neighborhood **chapters**, starting on the East Coast with one pilot in Brooklyn, NY. Small teams of trained Zendo volunteers — a lead responder plus up to two support responders — respond in person to people experiencing a mental-health crisis, a difficult psychedelic experience, or a distressing drug experience; every encounter has **remote licensed clinical oversight**; and each neighborhood has a quiet **Safe Zone** for de-escalation.

What makes it different

- Built on Zendo's Four Principles of Psychedelic Care and existing volunteer training.
- Peer-led, clinically supervised, medically backstopped — never a replacement for 911 or 988.
- A purpose-built responder app aligned with Zendo's existing **Serenity** documentation tool.
- An in-app on-call resource library plus one-tap access to the on-call LCSW / MD.
- Grows slowly, one chapter at a time, with go/no-go gates.

Each chapter includes

PAID / STIPEND

Chapter Coordinator

Runs scheduling, dispatch oversight, volunteer care, and partner relationships.

PAID

Remote Clinical Supervisor

LCSW (or LMSW under qualifying supervision) available live to every encounter.

PAID / ON-CALL

On-call Psychiatrist (MD/DO)

Consults on emergencies, medical/psychiatric red flags, and escalation.

STIPEND OR VOLUNTEER

Hospital & Clinic Liaison

Helps admitted guests and families navigate psychiatric EDs and clinics.

VOLUNTEER

Tier 2 Responders

Advanced-trained volunteers who lead field responses and Safe Zone shifts.

VOLUNTEER

Tier 1 Sitters

Foundation-trained volunteers who sit in the Safe Zone, at events, and as support responders on field teams.

Top-line asks of Zendo / MAPS leadership

1. **Permission to explore** — approval to develop this concept under Zendo's name with a named staff sponsor.
2. **Alignment on training** — confirm how Zendo's two training levels map to Tier 1 / Tier 2, and co-design a field-response module.
3. **Serenity access** — a conversation with Zendo's systems team about extending or integrating with Serenity.
4. **Legal & insurance review** — engage Zendo's counsel on liability, scope of practice, and privacy before any field activity.
5. **A 6-month discovery phase** — partner meetings in Brooklyn, a needs assessment, and a funded pilot budget.

FOUNDATION

2. Zendo Project today

Zendo Project began at Burning Man in 2012 as a harm-reduction initiative of MAPS. After ten years of incubation, it became its own 501(c)(3) nonprofit in 2023 while remaining "closely allied with MAPS."[1] Its current mission is "to expand compassionate crisis support — including care for non-ordinary psychological and emotional states — through training, peer-based services, and research across communities and systems of care."[2] The team includes mental-health professionals, doctors, nurses, social workers, and harm-reduction experts, with a Medical Director who is an emergency-medicine and EMS physician.[2]

2012
Founded at Burning Man as a MAPS program[1]

10,000+
trained peers and professionals (per Zendo)[3]

6,500+
people cared for across four continents in first 10 years[1]

416 / 566
volunteers trained / guests served, Burning Man 2022[1]

The Four Principles of Psychedelic Care[2]

Safe Space
Tending to safety is foundational to healing and transformation.

Sitting, Not Guiding
Cultivating presence while letting go of an agenda honors the individual's inner healing intelligence.

Through, Not Down
We accompany people through their experiences, without seeking to halt or bypass the process.

Difficult Is Not Necessarily Bad
Difficult and/or complex experiences can lead to wisdom, growth, and new understandings.

How Zendo works at events

Zendo provides on-site peer support and a "tranquil space" at events, working "collaboratively with production, medical, security, and law enforcement."[4] Volunteer roles include **Greeters**, **Sitters**, **Logkeepers** (who "keep track of lots of moving people and parts, using an iPad"), **Logistics**, **Medical Triage** (EMT or higher required), and **Pulse**, which supports volunteer well-being. All event volunteers receive pre-event and on-site training.[4] Zendo's early mission explicitly included reducing "the number of drug-related psychiatric hospitalizations" and demonstrating that safe experiences are possible "without the need for law enforcement-based policies."[5]

Training offerings (public)

- **Free Psychedelic Care Kit / mini-course** — a 30-minute training on the Four Principles plus a pocket card.[3]
- **Zendo SIT (Specialized Training in Psychedelic Care)** — 8 hours of live online training plus 1 hour self-paced; CE credit available for eligible licensed professionals.[6] Modules cover the Four Principles and "Navigating Critical Situations," including identifying medical and mental-health emergencies within the scope of peer support.[7]

The volunteer reports two Zendo certification levels; their exact names and requirements should be confirmed with Zendo: [Level 1 name] / [Level 2 name]. Important: Zendo states the SIT Course Completion Award "does not constitute ... authorization to provide ... crisis services."[6] This plan therefore adds a separate, supervised field-response credential (Section 11).

Serenity — Zendo's encounter documentation tool

A 2026 peer-reviewed article co-authored by Zendo-affiliated researchers describes **Serenity** as "Zendo Project's internal data collection tool," developed to standardize encounter documentation with time-stamped records of **intake, reassessment, intervention, and disposition**.[8]

- Evolved from paper; first software version in 2019 with one iPad per site run by a Logkeeper; later expanded to shift leads, medical triage, and coordinators.
- Validates required fields at guest departure, embedding data quality in the workflow.
- Designed so "sitters do not interact with devices while supporting guests."[8]

Why this matters: Serenity already encodes the encounter lifecycle this program needs. Building the community app *with* Serenity — rather than beside it — preserves Zendo's data model, quality standards, and its principle that technology should stay out of the moment of care.

CONTEXT

3. The need and the opportunity

Psychedelic and other substance use happens mostly outside clinics and festivals — at home, at parties, and in public space. When those experiences become frightening, people and their friends often have only two options: ride it out alone or call 911.

MAPS and Zendo have noted that over 5.5 million people in the U.S. used psychedelics in 2019 (Livne et al., 2022), and that "most psychedelic use occurs outside of therapeutic contexts."[1] Zendo's leadership has argued that its skills are "highly transferable" and could "fill a gap in care" by making peer support more available to everyone.[1] Local data for Brooklyn should be gathered during discovery:
[# psychiatric ED visits related to hallucinogens/stimulants, Brooklyn, most recent year — source: NYC DOHMH / SPARCS], [# 911 mental-health (EDP) calls in pilot precincts], [nightlife venue count in pilot area].

What existing models teach us

Model	What it is	Lesson for this program
CAHOOTS Eugene/Springfield, OR	Launched 1989 by White Bird Clinic; two-person teams (a medic plus a crisis worker), unarmed, using trauma-informed de-escalation and harm reduction. White Bird reported roughly 24,000 calls in a year with police backup requested only 150 times, on a ~$2.1M budget.[9]	**Non-police response works — and funding is fragile.** In April 2025 White Bird and the City of Eugene ended CAHOOTS service in Eugene over contract and funding issues; city funding covered about 40% of its cost there. Service continued in Springfield.[10] Grow slowly and diversify funding.

Model	What it is	Lesson for this program
B-HEARD New York City	Since 2021, teams of two FDNY EMTs/paramedics plus an NYC Health + Hospitals mental-health professional respond to eligible 911 mental-health calls, 16 hours/day in parts of four boroughs, including several Brooklyn neighborhoods. Teams are not dispatched where violence or imminent harm is identified.[11]	**A complement, not a competitor.** A 2025 Comptroller audit (as reported) found teams did not respond to over one-third of eligible calls; staffing and funding remain challenges.[12] Zendo chapters could fill the lower-acuity, substance-related, and event niche B-HEARD does not target.
988 Lifeline	National three-digit Suicide & Crisis Lifeline, launched July 2022, funded by SAMHSA and administered by Vibrant Emotional Health; 24/7 support for mental-health, suicide, and substance-use crises.[13,14] In NYC, 988 connects to counselors and the city's urgent mental-health services.[11]	**Built-in escalation path.** The app should offer one-tap 988 and 911, and the program should seek a referral relationship with NYC 988.
Fireside Project	Nonprofit Psychedelic Support Line (since 2021): free, confidential peer support by phone, text, and app, 11 a.m.–11 p.m. Pacific, at 62-FIRESIDE; ~40,000 conversations, 95% of callers felt heard and understood (self-reported).[15,16]	**Remote + in-person = continuum.** Fireside offers remote support; Zendo chapters could offer the in-person layer. Warm-handoff and integration-support partnership is a natural fit.
Peer crisis response	NYC Mobile Crisis Teams pair master's-level clinicians with peer staff for urgent (non-911) referrals.[11]	Peer + clinician pairing is an established practice — our model applies it with clinicians supervising remotely.

The gap this program fills

A person having a terrifying trip in a Brooklyn apartment at 2 a.m., or a friend sitting with someone at a warehouse party, needs someone calm, trained, and non-judgmental *now* — but often does not need police, an ambulance, or an inpatient admission. Zendo's model is purpose-built for exactly that moment. The opportunity is to offer it locally, year-round, with professional oversight and clear escalation paths.

DESIGN

4. Program model: chapters & roles

The program is organized as **neighborhood chapters** under an East Coast program umbrella, which itself sits under Zendo Project's governance, training standards, and medical direction. Chapters share one app, one training pathway, and one set of protocols; each chapter is small enough that volunteers can reach a person on foot or by short transit ride.

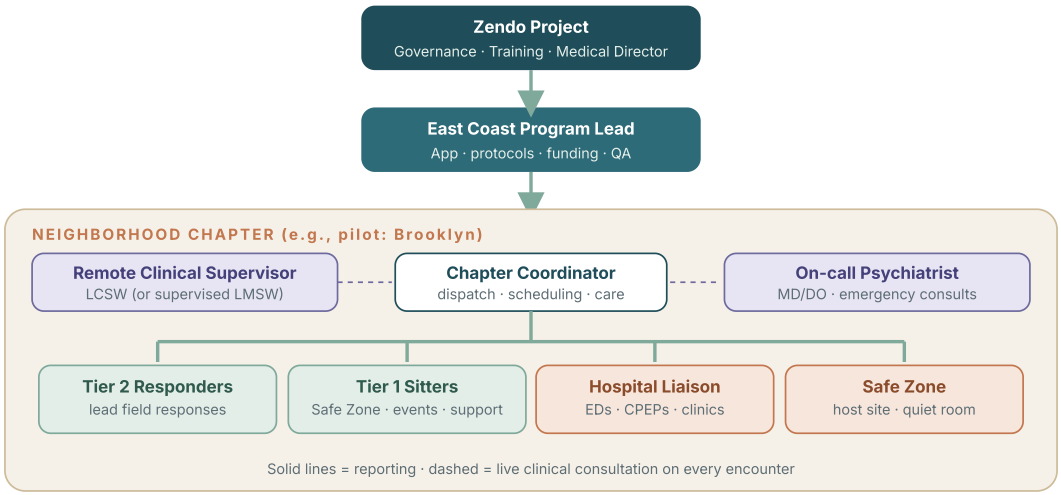


Figure 1. Proposed chapter structure.

Role	Core responsibilities	Qualifications	Status
Chapter Coordinator	Owns the on-call roster and event coverage; monitors live encounters in the app; recruits and supports volunteers; runs debriefs; maintains partner and Safe Zone relationships.	Zendo Tier 2; leadership experience; ideally harm-reduction background.	Paid part-time
Remote Clinical Supervisor	Joins every encounter by in-app chat/voice; supports risk assessment; decides with responder on escalation; signs off on encounter close; provides group supervision.	Licensed in the state where the guest is located — see scope note below.	Paid (hourly on-call + supervision)
On-call Psychiatrist	Second-line consult for medical/psychiatric red flags (e.g., suspected serotonin syndrome, psychosis with risk, interactions); advises when to call 911; reviews protocols with Zendo's Medical Director.	MD/DO, state-licensed; psychiatry or emergency medicine.	Paid retainer / per-consult
Hospital & Clinic Liaison	Maintains relationships with local EDs, psychiatric emergency programs, and clinics; supports guests/families (with consent) after admission; helps route to the right level of care.	Social-work, peer-specialist, or patient-navigator background.	Stipend or volunteer [decide]
Tier 2 Responder	Lead responder on field teams (or support when another lead is assigned); leads Safe Zone shifts; mentors Tier 1.	Zendo Level 2 [confirm] + field credential.	Volunteer
Tier 1 Sitter	Sits with guests in Safe Zones and at events; support responder on field teams led by a Tier 2.	Zendo Level 1 [confirm] + orientation.	Volunteer

Clinical oversight: LMSW vs. LCSW scope (verify with counsel)

- **New York:** an LMSW may provide clinical social work services only under supervision by a qualified LCSW, psychologist, or psychiatrist; an LCSW may practice clinical social work independently.[17,18]
- **New Jersey:** an LSW may provide clinical services only under supervision of an eligible LCSW (including at least one hour weekly of face-to-face or qualifying HIPAA-compliant video supervision).[19]
- **Massachusetts:** the independent clinical license is the **LICSW**; an LCSW may do clinical work only when employed under LICSW supervision.[20]

Recommendation: make the chapter's lead clinical supervisor an independently licensed clinician (NY/NJ LCSW, MA LICSW). LMSWs can staff on-call shifts under that person's documented supervision. Counsel should advise whether real-time oversight of peer volunteers creates a clinical relationship with the guest, and how licensure applies when supervising remotely across state lines.

OPERATIONS

5. Encounter workflow

Every encounter follows the same lifecycle Zendo already documents in Serenity — **intake → reassessment → intervention → disposition**[8] — with a safety screen at the front and a clinician present (remotely) throughout.

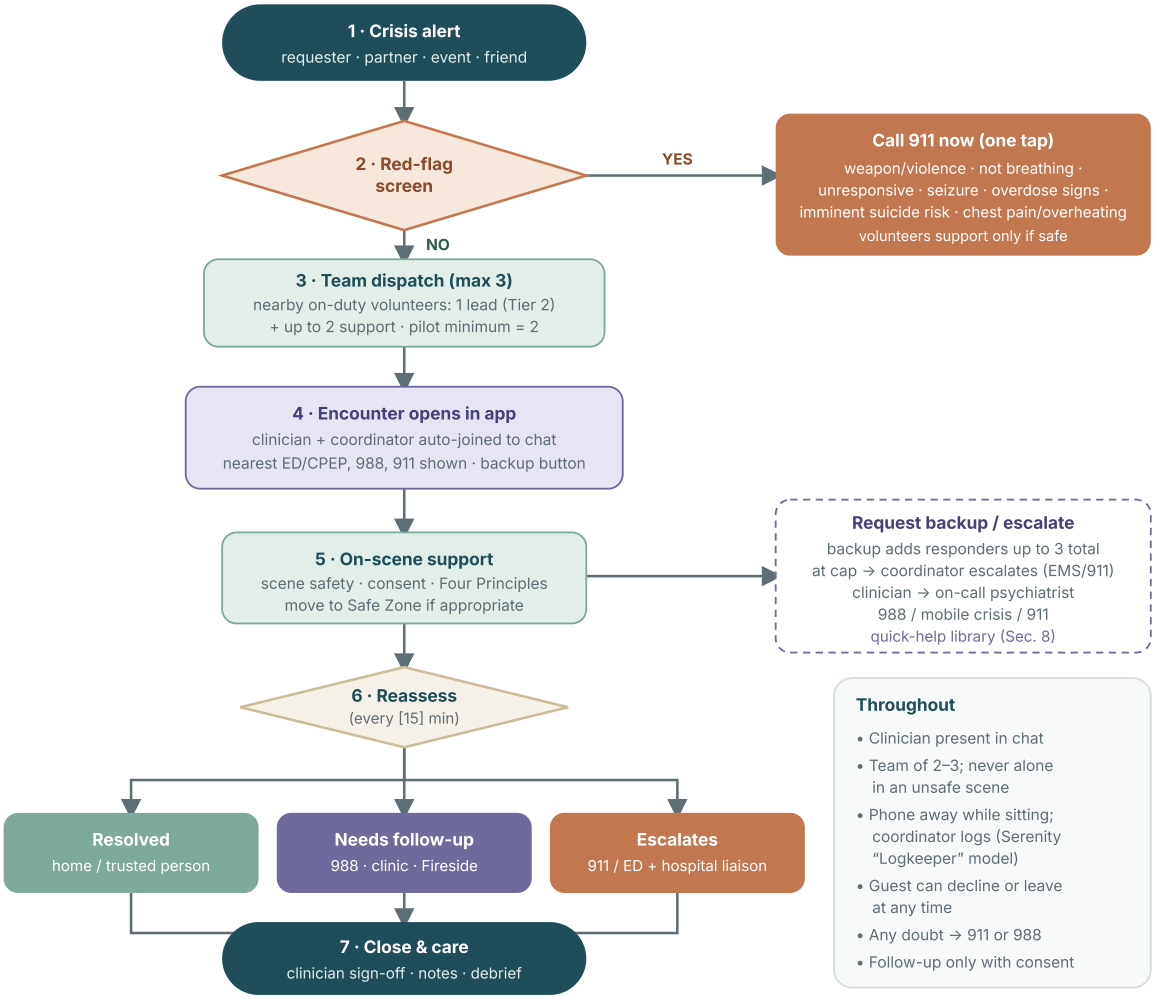


Figure 2. Proposed encounter workflow. Reassessment interval is a placeholder to be set by the clinical lead.

Step	What happens	Who
1 · Alert	A request arrives via the app (self, friend, venue, event staff, or partner referral). Requester sees: *"If anyone is in danger, call 911 now."*	Requester; app
2 · Red-flag screen	3–5 plain-language questions. Any "yes" → app prompts 911 and alerts coordinator; volunteers may support from a safe distance until EMS arrives.	Requester / coordinator
3 · Team dispatch	The app offers the call to nearby on-duty volunteers until a team forms: **one lead responder (Tier 2) plus up to two support responders** (Tier 1 or Tier 2) — **maximum 3**. Pilot minimum is a pair (lead + 1 support); if no pair can form within [X min], the coordinator decides next steps (remote support via 988/Fireside, wait, or EMS). ETA shared with requester.	Coordinator; responders
4 · Encounter opens	Secure encounter room auto-adds the on-shift clinician and coordinator; shows nearest ED/psychiatric emergency program, 988, 911, Safe Zone, and a "request backup" button.	App
5 · On-scene	Scene safety check, consent, basic needs (water, warmth, quiet), Zendo-style sitting. Move to Safe Zone if guest agrees and is safe to walk/travel. **"Request backup" adds responders up to the cap of 3;** if more help is needed at the cap, the request escalates to the coordinator, who calls EMS/911 or the clinician-advised next step.	Responders; clinician by chat/voice
6 · Reassess	Structured check-ins at set intervals; clinician confirms whether to continue, refer, or escalate.	Responders + clinician
7 · Disposition & close	Home/trusted person, referral, or ED with liaison notified. Clinician sign-off; brief notes; responder debrief and Pulse-style check-in.	All

Team roles on scene (up to 3)

Lead responder (Tier 2)
Primary sitter; holds the relationship with the guest; makes decisions with the remote clinician; the only one who needs to speak much.

Support 1 (Tier 1 or 2)
Scene safety and environment (light, noise, space); talks with friends/bystanders; keeps the coordinator updated and uses the app so the lead doesn't have to.

Support 2 (on backup)
Second sitter or relief; fetches supplies; accompanies the move to a Safe Zone; meets EMS at the door. Beyond three → coordinator/EMS, not more volunteers.

Why a cap of 3: more people can overwhelm a frightened person and undercut *Safe Space* and *Sitting, Not Guiding*; needing more than three people is itself a sign the situation may need professional or emergency services.

PLACE

6. Neighborhood Safe Zones

A Safe Zone is the year-round equivalent of the Zendo space at an event: a quiet, warm, low-stimulation room where a guest and volunteer can sit, rest, and let the experience settle. The aim is the first principle — *Safe Space* — made permanent in a neighborhood.

Proposed space requirements

- Private, quiet room(s) with dimmable light; space for 2–4 guests plus sitters [target sq ft]
- Comfortable seating, mats/beds, blankets, water, light snacks, art supplies (used since MAPS' early event safe spaces[21])

Hosting options (lowest cost first)

1. **Partner-hosted room** in an existing harm-reduction, community, or faith space, after hours [in-kind]
2. **Pop-up Safe Zone** at events and parties (Zendo's existing model)
3. **Shared lease** with an allied nonprofit

- Accessible entrance, restroom, climate control, Wi-Fi/cell signal
- First-aid kit, AED [confirm], naloxone, posted emergency plan
- Separate staff area for debriefing; secure storage; no surveillance cameras in care areas
- Clear address for EMS access

4. **Dedicated storefront** (later phases only)

Staffing during open hours: at least one Tier 2 + one Tier 1 on site, coordinator on call, clinician reachable in-app. Open hours start limited (e.g., weekend nights) and expand with demand data.

Safe Zone costs (placeholders — obtain quotes)

Item	Basis	One-time	Annual
Space (partner-hosted or lease)	Hours/week × rate, or in-kind MOU	—	[$ — / in-kind]
Build-out & furnishing	Soft furnishings, lighting, soundproofing, signage	[$ —]	—
Safety equipment	First-aid, naloxone, AED, radios/phones	[$ —]	[$ — restock]
Supplies & hospitality	Water, snacks, blankets, laundry, cleaning	—	[$ —]
Insurance rider for premises	Broker quote	—	[$ —]

Outcomes from a Safe Zone

Resolved → guest leaves with a friend/family member or is accompanied home (transport policy TBD). **Needs ongoing support →** warm referral (988, clinic, integration support such as Fireside). **Escalates →** 911/ED, with the hospital liaison notified so the guest and family are not left to navigate the system alone.

TECHNOLOGY

7. The responder app

The app is the program's nervous system: it routes alerts, opens a supervised encounter, keeps escalation one tap away, and lets volunteers sign up for events. It should feel calm, minimal, and usable by someone who is nervous and in the dark.

Responder

- On-call toggle & availability
- Nearby alert → accept as lead or support
- Team view: who's coming, roles, ETAs
- Encounter room: chat/voice with clinician & coordinator
- "Request backup" — adds responders up to 3; at cap becomes "Escalate to coordinator / EMS"
- On-call resource library & "Ask LCSW / MD" (Sec. 8)
- One-tap 911 / 988; nearest ED, psychiatric ER, Safe Zone
- Post-encounter notes & debrief prompt
- Event & party shift sign-ups

Coordinator / Clinician

- Live map of on-duty responders (on shift only)
- Team-fill status per call (1/3 · 2/3 · 3/3) and backup requests
- Encounter dashboard & timers for reassessment
- Escalate to on-call psychiatrist
- Clinician sign-off to close encounter
- Roster, events, coverage gaps
- De-identified reports

Requester (later phase)

- "I need support" / "My friend needs support"
- Red-flag screen → 911 prompt
- Location share (session only)
- ETA & responder first name
- Grounding content while waiting
- Optional anonymous feedback

MVP scope (pilot) — build the smallest thing that is safe

Feature	MVP?	Notes
Volunteer onboarding, roles, credential status	Yes	Only credentialed Tier 2 can accept as lead; Tier 1 may accept as support.
Alert intake by coordinator	Yes	Pilot alerts arrive via partners/hotline and are entered by the coordinator — no public-facing request button yet.
Team dispatch (lead + up to 2 support)	Yes	Offers the call to nearby on-duty volunteers by distance until a lead and ≥1 support accept; hard cap of 3; alerts coordinator if a pair can't form in [X min]. Location used only while on duty; coarse location stored.
Encounter room (secure chat + voice)	Yes	Clinician + coordinator auto-added; end-to-end encrypted.
Backup (to cap of 3) / 911 / 988 / nearest facilities	Yes	Backup fills open team slots first; at cap, routes to coordinator/EMS. Static, verified facility list per chapter.
On-call Volunteer Resources library	Yes (core)	Quick cards, checklists, scripts available offline; "Ask LCSW / MD" button (Section 8).
Serenity-aligned encounter record	Yes	Intake → reassessment → intervention → disposition; required fields at close.
Event / party shift sign-ups	Yes	Could reuse an existing volunteer-scheduling tool initially.
Resource Board	Phase 2	See Section 9.
Public requester app	Phase 3+	Only after protocols, insurance, and capacity are proven.

Integration with Serenity

- **Option A — extend Serenity** with a "community encounter" mode (preferred if Zendo's systems team agrees).
- **Option B — separate dispatch app** that writes encounter records into Serenity's schema.
- Keep Serenity's design rule: sitters don't use devices while supporting guests; the **remote coordinator acts as Logkeeper**, and responders complete notes afterward.[8]
- Reuse Serenity's required-field validation at disposition.

Privacy & HIPAA posture

- **HIPAA** applies to "covered entities" (health plans, clearinghouses, and providers that conduct certain electronic transactions such as billing) and their business associates.[22] Whether this program is covered depends on its structure — counsel to decide. Recommendation: **design to HIPAA-grade standards regardless** and sign BAAs with vendors.
- **42 CFR Part 2** may apply to federally assisted substance-use-disorder programs.[23] NY's **SHIELD Act** sets data-security and breach-notice duties.[24]
- Data minimization: no names required; no substance info tied to identity; location purged after close; retention schedule [e.g., X days].
- Written policy for law-enforcement data requests; informed consent language reviewed by counsel.

IN-THE-MOMENT SUPPORT

8. On-call Volunteer Resources

A quick-help library inside the app for the volunteer who is on a call right now: short, clinically approved, findable in two taps from the encounter room, and usable offline.

Consistent with Serenity's design principle that sitters do not use devices while supporting guests,[8] the library is meant to be glanced at — typically by a **support responder** while the lead stays present with the guest — or reviewed on the way to a call.

Training refreshers

2–5 minute refreshers drawn from Zendo SIT and the free Four Principles training (the 30-minute course in Zendo's Psychedelic Care Kit).[3,6] [content license from Zendo]

Principles quick cards

The Four Principles[2] with "try saying / avoid saying" examples, building on SIT skills such as normalizing, validation, curiosity, and reframing.[7]

De-escalation guides

Adjusting the environment, grounding and breathing, compassionate boundaries, letting go of power struggles,[7] working with worried friends.

Red-flag & escalation checklists

The 911 and clinician triggers from Section 12 as tap-through checklists, with one-tap 911/988 and the "team at cap → coordinator" path.

Substance quick refs

Typical onset/duration, warning signs (e.g., overheating, serotonin toxicity, opioid overdose), risky combinations. Written and reviewed by the Medical Director / psychiatrist; **no dosing advice**.
[source references TBD]

Crisis scripts

Opening and consent, "you're safe, I'm here," talking to bystanders, calling 911 (with the Good Samaritan reminder[25]), handing off to EMS, closing and follow-up consent.

Ask the on-call LCSW / MD

One button from any card: chat or voice with the on-shift clinician, who can loop in the on-call psychiatrist. Logged in the encounter record.

Local quick list

Nearest ED / psychiatric emergency program, Safe Zone address and door code, 988, Fireside (62-FIRESIDE, 11 a.m.–11 p.m. PT).[15]

Governance: every item is approved by the clinical lead (and Medical Director for medical content), versioned, stamped "last reviewed," and re-reviewed every [6] months. The library supports training; it does not replace it or authorize anything beyond a volunteer's tier.

How this differs from the Community Resource Board

	On-call Volunteer Resources (Section 8)	Community Resource Board (Section 9)
Purpose	Help a volunteer handle the call in front of them	Volunteers ask each other for local resources (services, clinics, language support)
Content from	Program, Zendo training, clinicians, Medical Director	Volunteers — posts and replies
When used	During or right before an encounter	Between encounters; planning and follow-up
Quality control	Clinically approved before publishing; versioned	Moderated; useful answers verified into a directory
Live help	"Ask LCSW / MD" — real-time clinical consult	Peer replies; not for emergencies
Phase	MVP (core cards) in the pilot	Phase 2

COMMUNITY

9. Community Resource Board

Every volunteer carries knowledge: a clinic that sees uninsured patients, a Spanish-speaking integration circle, a shelter with open beds tonight, a lawyer who helps with ACS cases. The Community Resource Board turns that scattered knowledge into a shared, living network. (For in-the-moment help on a call, see the On-call Volunteer Resources in Section 8.)

How it works

1. A volunteer posts a need ("Need: Haitian Creole-speaking peer support near Flatbush").
2. Any volunteer can reply, upvote, or tag someone who knows.
3. Helpful answers are saved to a **verified chapter directory** after coordinator review.
4. Directory entries carry "last verified" dates and are re-checked quarterly.

Guardrails

- **No guest-identifying information** in posts — needs are described generically.
- Not for emergencies (auto-reminder: use the encounter room / 911 / 988).
- Moderated by coordinators; code of conduct.
- Pilot can start in a moderated group channel or form before building it into the app.

10. Hospital & clinic liaison program

When an encounter ends in the emergency department, the person often enters a confusing system at their most vulnerable. Each chapter's liaison helps make that transition humane and helps guests reach the right level of care.

Liaison responsibilities

- Know local options: hospital EDs, Comprehensive Psychiatric Emergency Programs (CPEPs), crisis stabilization/support-and-connection centers, outpatient clinics, detox and integration resources
- Build relationships with ED social work and psychiatric intake teams; pursue MOUs
- With the guest's consent, follow up after admission: help family understand the process, pass on (with consent) the de-identified encounter summary, connect to discharge resources
- Feed learnings back to the resource directory

Boundaries

- Liaisons are navigators and advocates, not treating providers
- All contact with hospitals about a guest requires the guest's written or recorded consent (or a lawful basis advised by counsel)
- Hospital access rules govern any on-site presence
- Pilot targets: [NYC Health + Hospitals/Kings County — verify CPEP], [other Brooklyn ED], [community clinic]

Context: NYC B-HEARD teams offer everyone served follow-up care, including mobile crisis, outreach, or hospital-based programs;[11] NYC has one Support and Connection Center in East Harlem offering short stays as a hospital alternative (as reported in 2026).[12] The liaison should map which of these a peer program can refer to.

PEOPLE

11. Training & certification pathway

The pathway builds on Zendo's existing training and adds field-specific skills that event work doesn't require (responding in homes and streets, working remotely with a clinician, urban safety). Final content should be co-designed with Zendo's training team and Medical Director.

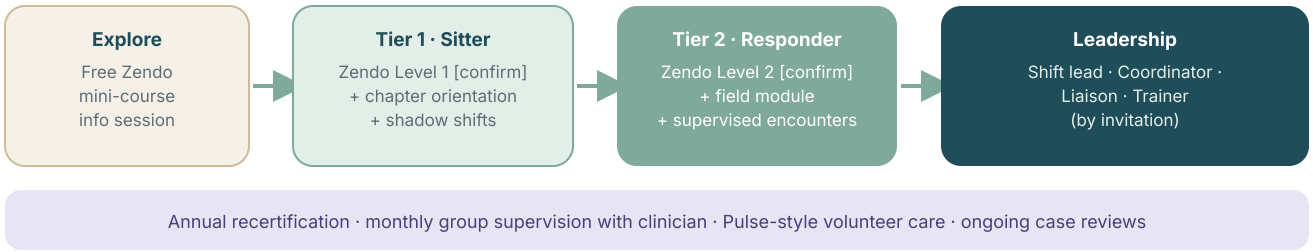


Figure 3. Proposed volunteer pathway.

Component	Tier 1 — Sitter	Tier 2 — Responder
Zendo foundation	Zendo Level 1 [confirm name/hours]	Zendo Level 2 [confirm]; Zendo SIT recommended[6]
Safety skills	CPR/First Aid; naloxone (opioid overdose) training; basic de-escalation	All Tier 1 + situational/urban safety; recognizing medical red flags (with Medical Director); suicide-risk screening under clinician direction [tool TBD by clinical lead]

Component	Tier 1 — Sitter	Tier 2 — Responder
Program protocols	Safe Zone procedures; consent; confidentiality; mandated-reporting awareness [counsel]	Field response; working with remote clinician; escalation & 911 handoff; documentation
Practicum	[#] shadow shifts in Safe Zone or at events	[#] encounters as support responder + sign-off by clinical lead before leading
Screening	Application, interview, references, background check [scope per counsel]	Same + demonstrated reliability at Tier 1 for [# months]
May do	Sit in Safe Zone/events; support responder on a field team led by a Tier 2	Accept field alerts as lead or support; lead Safe Zone shift; mentor Tier 1

Note: this program credential would be issued by the program (and ideally co-branded by Zendo once approved). It is distinct from the Zendo SIT Course Completion Award, which Zendo states is not a credential or authorization to provide crisis services.[6]

SAFETY

12. Safety, escalation & risk protocols

This program is designed for **non-violent, non-life-threatening** situations. It does not replace 911, EMS, or 988, and responders never trade safety for presence. Final protocols must be written and signed off by the clinical lead and Zendo's Medical Director.

Call 911 immediately when…

- Anyone is unresponsive, not breathing normally, or has blue/grey lips (give naloxone if opioids possible)
- Seizure, chest pain, very high body temperature, severe confusion with fever/rigidity
- Serious injury, head injury, or uncontrolled bleeding
- Weapon present, violence, or threat of violence
- Imminent risk of suicide or harm to others
- Any team member feels unsafe for any reason

New York's 911 Good Samaritan law (Penal Law §220.78) protects people who seek emergency help for an overdose from certain possession charges; it has limits (e.g., sale offenses, warrants, parole/probation).[25] Volunteers should know and share this to reduce fear of calling.

Call the clinician / on-call psychiatrist when…

- Any mention of suicidal thoughts or self-harm (clinician leads risk assessment)
- Psychotic symptoms that persist or include risk
- Possible dangerous combinations (e.g., stimulants, MAOIs/SSRIs with MDMA, alcohol + sedatives)
- The person is a minor [minors policy — counsel]
- Guest wants to leave but may be unsafe
- No improvement after [X] reassessments
- The team is at its cap of 3 and still needs more help (coordinator escalates)

Volunteer safety standards (proposed)

Teams of 2–3

Each call gets a Tier 2 lead plus up to two support responders (cap 3). Pilot minimum is a pair; no one enters a private residence alone. Backup fills the team to 3; beyond that, the coordinator escalates to EMS.

Scene check

Pause before entering; coordinator knows exact location; check-in timer; code word for "I need help." Leave and call 911 if unsafe.

No transport

Volunteers don't drive guests in personal vehicles in the pilot; walk, rideshare with a partner, or EMS [policy].

Boundaries

No restraint, no medication, no substances, no "guiding" the trip. Zendo's non-directive approach applies.

Care for carers

Mandatory debrief after each encounter; Pulse-style peer support; access to counseling; shift limits.

Incident review

Every escalation and safety incident gets a blameless review with the clinical lead within [7] days.

13. Legal, liability & insurance

The points below are issues to raise with counsel, not legal conclusions.

Issue	What we know	Action
Good Samaritan (civil)	NY Public Health Law §3000-a shields people who voluntarily and without expectation of payment give first aid or emergency treatment outside a medical facility, except for gross negligence; it excludes licensed health professionals acting in their professional capacity.[26]	Ask counsel whether it applies to an organized, pre-arranged program (likely limited) and to paid staff.
Volunteer Protection Act	Federal law (42 U.S.C. §14501 et seq.) limits some liability for nonprofit volunteers acting within scope; it does not protect the organization itself.[27]	Clarify scope, training, and documentation needed; state variations (NY/NJ/MA).
Scope of practice	Peer support is not therapy or medical care. LMSW/LCSW and MD roles are governed by state licensure rules (Section 4).[17–20]	Written scope statements per role; clinician contracts; cross-state licensure plan.
Drug-related exposure	Volunteers encounter people who have used controlled substances; harm-reduction programs must avoid any appearance of facilitating use.	Clear no-possession/no-distribution policy; counsel review of Safe Zone operations.
Privacy & records	HIPAA, 42 CFR Part 2, SHIELD Act may apply (Section 7).[22–24]	Privacy impact assessment before app build.
Mandated reporting / minors	Obligations vary by role and state.	Policy for minors and child/elder-abuse disclosures.

Insurance to quote (via a nonprofit broker)

General liability · professional liability / errors & omissions (covering peer services and clinical consultants) · abuse & molestation · directors & officers · cyber liability (app data) · volunteer accident · hired/non-owned auto · premises coverage for Safe Zones. Confirm whether Zendo's existing event coverage can be extended or whether a separate policy is needed: [annual premium estimate].

RESOURCES

14. Staffing & budget (pilot chapter, year 1)

All dollar figures below are **placeholders**. Hours are proposed planning assumptions for a pilot with limited operating hours; they should be replaced with local salary benchmarks (e.g., BLS wage data, partner pay scales) and vendor quotes during discovery.

Line item	Paid / volunteer	Basis (proposed)	Low	High
Personnel				
Chapter Coordinator	Paid (PT)	~20 hrs/wk × hourly rate	[$]	[$]
Remote Clinical Supervisor(s)	Paid	On-call hours during operating window + 2 hrs/mo group supervision	[$]	[$]
On-call Psychiatrist	Paid	Monthly retainer + per-consult fee	[$]	[$]
Hospital & Clinic Liaison	Stipend / vol.	~5–10 hrs/wk	[$0]	[$]
Tier 1 & Tier 2 volunteers	Volunteer	Sized by ratio assumptions below	$0	$0
East Coast Program Lead (shared)	Paid / vol.	Share of time allocated to pilot	[$]	[$]
Training				
Zendo training seats, CPR, naloxone, field module	—	Per-volunteer cost × cohort; scholarships	[$]	[$]
Background checks	—	Per volunteer	[$]	[$]
Technology				
App MVP build (or Serenity extension)	—	Vendor quote or pro-bono partner	[$]	[$]
Hosting, secure messaging, licenses	—	HIPAA-eligible vendors with BAAs	[$]	[$]
Safe Zone		See Section 6	[$]	[$]
Insurance		Broker quote (Section 13)	[$]	[$]
Supplies & equipment		Kits, naloxone, phones/radios, IDs	[$]	[$]
Evaluation		Data analysis; optional academic partner	[$]	[$]
Admin / fiscal overhead		[%] of direct costs	[$]	[$]
Total pilot year			**[$ total]**	**[$ total]**

Volunteer-ratio assumptions (planning, to validate in pilot)

- **Per call:** 1 lead (Tier 2) + 1–2 support; pilot minimum 2, cap 3.
- **On duty per operating shift:** (expected simultaneous calls × 3) + [1] reserve, with at least [2] lead-eligible Tier 2s — e.g., one call at a time → about 4 on duty.
- **Active roster:** shifts per month × people on duty ÷ shifts each volunteer commits to per month, plus an attrition buffer of [30%].
- **Tier mix:** about [1 Tier 2 : 1–2 Tier 1]; Safe Zone shifts need 1 Tier 2 + 1 Tier 1 on site.
- **Clinician load:** one remote clinician oversees up to [X] simultaneous encounters (set by the clinical lead); add a second clinician above that.

Illustration only: 8 shifts/month × 4 on duty = 32 shift-slots; at 2 shifts per volunteer per month that is 16 active volunteers, or about 21 with a 30% buffer. Real inputs come from pilot demand and volunteer commitments.

Why pay the clinicians?

Real-time clinical oversight of every encounter is the program's core safety promise, and it must be reliable at 2 a.m. Volunteer clinicians are hard to schedule consistently, so this plan assumes paid on-call clinical coverage from day one, with volunteer clinicians as a supplement. Many clinicians may also value supervision hours and CE-relevant experience.

15. Funding strategy

CAHOOTS' 2025 contraction in Eugene, where city funding covered ~40% of costs and federal grants became uncertain,[10] is a reminder to avoid single-source dependence.

Philanthropy

Psychedelic-aligned foundations and donors (including those who already support Zendo and MAPS); community foundations in Brooklyn/NYC; individual giving and a "sponsor a Safe Zone" campaign.

Public funds

NYC DOHMH and NYC Council discretionary funding; New York State OMH / OASAS grants; opioid settlement funds (harm-reduction eligible uses — verify); SAMHSA crisis-continuum grants.

Earned & in-kind

Event service contracts (festivals, nightlife venues); trainings for venues and staff; pro-bono app development; donated space; university partnerships for evaluation.

ECOSYSTEM

16. Partnerships

Partner	Why	Proposed ask
Zendo Project / MAPS	Brand, principles, training, Serenity, Medical Director, event network	Sponsorship of pilot; training co-design; data & tech alignment
Fireside Project	National psychedelic peer support line (phone, text, app), 11 a.m.–11 p.m. PT[15]	Two-way warm handoff protocol; integration follow-up; shared learning; possible cross-training
NYC 988 / 988 Lifeline	24/7 crisis counseling and access to NYC's urgent mental-health services[11,14]	Referral pathway for after-hours or remote support; awareness of chapter coverage
NYC DOHMH & Mayor's Office of Community Mental Health	Oversee crisis programs including B-HEARD and mobile crisis[11]	Briefing; data sharing on needs; potential funding; avoid duplication

Partner	Why	Proposed ask
Hospitals & CPEPs (e.g., NYC Health + Hospitals)	Escalation destination; liaison program	MOU for liaison contact, handoff format, and referral back to community
OnPoint NYC and other harm-reduction orgs	OnPoint operates two overdose prevention centers (East Harlem, Washington Heights) and reports 2,000+ overdose interventions[28]	Cross-training (naloxone, overdose response); referrals; possible Safe Zone hosting by local harm-reduction partners
Nightlife & event producers	Where many difficult experiences start	Event coverage contracts; venue staff training; referral line
Universities	Evaluation, volunteers (social work, nursing, medicine)	Field placements; IRB-approved evaluation (as in Zendo's 2026 study)[8]

17. Pilot plan & phased rollout

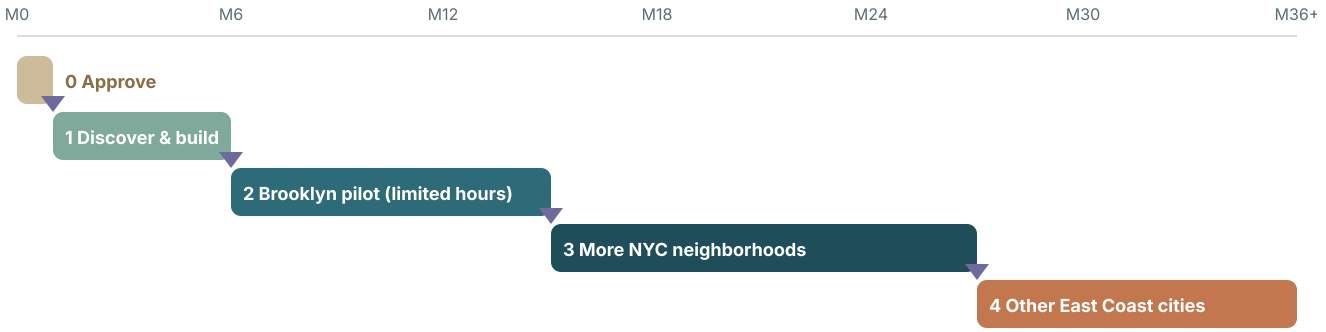


Figure 4. Indicative timeline; each ▼ marks a go/no-go gate reviewed by Zendo leadership. Months are planning assumptions.

Phase	Key activities	Gate to move on
0 · Approval ~Month 0–1	Present plan; name Zendo sponsor; agree scope, branding, and decision rights	Written go-ahead for discovery
1 · Discover & build ~Months 1–6	Brooklyn needs assessment & partner meetings; counsel and insurance review; hire clinical lead; draft protocols; recruit first cohort; app MVP or Serenity extension; secure Safe Zone host; pilot funding	Insurance bound; protocols signed by clinical lead & Medical Director; ≥ [#] credentialed Tier 2; funding for 12 months
2 · Brooklyn pilot ~Months 6–15	Start with event coverage and Safe Zone on [e.g., Fri–Sat nights]; then partner-referred field responses; monthly review of every escalation	Safety record meets thresholds; volunteer retention; partner satisfaction; sustainable funding path
3 · More NYC chapters ~Months 15–27	Add 1–2 neighborhoods [candidates]; regional clinical pool; Resource Board; consider public requester feature	Chapter playbook validated; multi-year funding
4 · East Coast ~Month 27+	Replicate in cities with strong volunteer bases [e.g., Philadelphia, Boston, New Jersey, DC — to be assessed]; state-specific licensure and legal review	Local anchor partner + licensed clinical lead in that state

18. Success metrics

Reach & responsiveness

- Encounters per month (field, Safe Zone, events)
- Median time from alert to arrival — target [X min]

Safety & quality

- Adverse events and safety incidents (target: zero serious harm to guests or volunteers)

- % of operating hours with full coverage (responders + clinician)
- % of calls reaching the minimum team of 2; time to full team; backup requests and at-cap escalations
- Events covered

Outcomes

- % resolved without ED transport; % appropriately escalated (clinician-reviewed)
- Police involvement rate
- Guest-reported experience ("felt safe/heard") — a measure Fireside also uses[15]
- % of consenting guests connected to follow-up
- % encounters with clinician present and signed off (target: 100%)
- Record completeness at close (Serenity-style validation)

Volunteers & sustainability

- Volunteers trained, credentialed, retained at 6/12 months
- Volunteer well-being check-ins
- Funding diversity (no source > [40]% of budget)
- Partner MOUs signed

Baselines for comparison (e.g., local ED visits or 911 mental-health calls) should come from public data or partners: [baseline sources]. Evaluation should follow the documentation and ethics approach Zendo used in its 2026 study.[8]

DECISIONS

19. Open decisions, next steps & asks

Key decisions for leadership

#	Decision	Options / recommendation
1	Program home & brand	Zendo-run program · Zendo-licensed affiliate · independent partner trained by Zendo. *Rec: Zendo-sponsored pilot with a clear brand-use agreement.*
2	Tier mapping	Confirm Zendo's two levels and what extra field training Tier 2 needs. *Rec: co-design with Zendo training team.*
3	Team size & composition	Up to 3 per call (lead + up to 2 support). Open: can Tier 1 be the only support? What happens if only one volunteer is available (wait, remote support via 988/Fireside with coordinator, or EMS)? Different cap at events? *Rec: minimum pair, cap 3, in the pilot.*
4	Who can raise an alert	Partners/events only · hotline referrals · public app. *Rec: partners + events first; public later.*
5	Clinical coverage model	One LCSW lead + LMSW on-call pool; shared across chapters; hourly vs retainer. Psychiatrist retainer; Medical Director role.
6	Safe Zone model & hours	Partner-hosted vs. leased; which nights/hours first.
7	App approach	Extend Serenity · new app writing to Serenity · off-the-shelf dispatch tool for pilot.
8	Liaison compensation	Volunteer vs. stipend vs. part-time staff.
9	Policies needing counsel	Transport, minors, mandated reporting, data retention, law-enforcement requests.
10	Pilot neighborhood	Which Brooklyn area? Weigh nightlife density, volunteer base, partners, and overlap with B-HEARD coverage areas.[11]
11	Fiscal home	Zendo budget line · fiscal sponsor · separate entity later.
12	On-call resource library content	Permission to adapt Zendo SIT / Four Principles material into in-app refreshers; who writes and approves substance quick refs.

Next 30 days

- Share this draft with Zendo leadership
- Confirm training levels and Serenity contacts
- List Brooklyn volunteers interested
- Informal calls: Fireside, a harm-reduction partner

Next 60 days

- Counsel & broker conversations
- Identify clinical lead candidates
- Shortlist Safe Zone hosts
- Gather local baseline data

Next 90 days

- Finalize pilot budget (replace placeholders)
- Submit first grant / donor asks
- Draft protocols v1
- App approach decision & prototype

What I'm asking for today

(1) A green light to run Phase 1 discovery under Zendo's guidance; (2) a named Zendo staff sponsor; (3) introductions to Zendo's training lead, Medical Director, systems/Serenity team, and counsel; and (4) permission to begin partner conversations in Brooklyn describing this as a *proposal under consideration*.

Disclaimer: volunteer-authored proposal; not affiliated with or endorsed by Zendo Project or MAPS until formally approved; not legal, medical, or clinical advice; highlighted placeholders must be replaced with verified data.

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